



NAME : Mrs.LILLY RICHARD	UHID NO/Visit ID : KAD393.00000002/KAD393.2
Age/Gender : 60 Y 0 M 0 D /F	Collected : 17/Jul/2026 01:33PM
Ref Doctor : SELF	Received : 17/Jul/2026 01:35PM
Ref. Cust : TEZ HEALTH	Reported : 17/Jul/2026 02:44PM
Client Code : KAD393	Barcode : H5668509

Test Name	Result	Unit	Bio. Ref. Range	Method
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COMPLETE BLOOD COUNT (CBC) - 25 TESTS

Haemoglobin	14.0	g/dL	12.0 - 15.0	Spectrophotometry(Cyanide Free)
Total WBC Count	4280	Cells/cumm	4000 - 10000	Electrical Impedance
RBC Count	4.86	Millions/cumm	3.8 - 4.8	Electrical Impedance
Platelet Count	217	10 ³ /μL	150 - 450	Electrical Impedance
Packed Cell Volume (PCV)	42.4	%	36 - 46	Calculated
Mean Corpuscular Volume (MCV)	87.2	fL	83 - 101	Calculated
Mean Corpuscular Hb. (MCH)	28.7	pg	27 - 32	Calculated
MCHC	33.0	g/dL	31.5 - 34.5	Calculated
MPV	8.5	fL	7 - 11	Calculated
Platelet Crit	0.184	%	0.15 - 0.62	Calculated
RDW CV	14.3	%	11.5 - 14.5	Calculated
RDW-SD	43.40	fL	39.5 - 46.0	Calculated
PDW	16.1	fL	11 - 22	Calculated
P-LCR	16.5	%	18 - 50	Calculated

Differential Count by Flowcytometry/Microscopy

Neutrophils	41.4	%	45 - 75	Impedance/microscopy
Lymphocytes	50.8	%	20 - 40	Impedance
Eosinophils	0.5	%	1 - 6	Impedance/microscopy
Monocytes	6.6	%	2 - 10	Impedance/microscopy
Basophils	0.7	%	0 - 2	Impedance/microscopy
Absolute Neutrophil Count	1771.92	cells/cumm	2000 - 7000.	Calculated
Absolute Basophils Count	29.96	Cells/cumm	0 - 100	DHSS with Impedance
Absolute Lymphocyte Count	2174.24	Cells/cumm	1000 - 3000	Calculated
Absolute Eosinophil Count	21.4	Cells/cumm	50 - 500	Calculated
Absolute Monocyte Count	282.48	Cells/cumm	200 - 1000	Calculated

Interpretation Notes:-

A complete blood count gives information regarding the cell types in person's blood and concentration of haemoglobin. Cells that circulate in the blood are generally divided into three types: RBC, WBC and platelets. Abnormally high or low counts may occur in physiological conditions and in diseased states and requires clinical correlation. Differential counts and RBC indices help in further understanding of the likely aetiology.

NOTE: This report has been generated by a fully automated analyser after counting thousands of cells and hence differential count may appear as decimalized numbers.


Dr. Nithyanandam Vinayagam
Consultant Pathologist





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C-REACTIVE PROTEIN (CRP) , SERUM

CRP	14.50	mg/L	< 5.0	Immunoturbidimetry
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Interpretation Notes:-

C - Reactive Protein (CRP) is an acute phase protein that is involved in the activation of complement, acceleration of phagocytosis and detoxification of substances released from damaged tissue. As such, CRP is considered to be one of the most sensitive indicators of inflammation. In response to an inflammatory stimulus, a rise in CRP may be detected with 6 hours. CRP is sensitive, though considered to be a non-specific indicator of acute phase reactants.

Measurement of C - Reactive Protein is most frequently used for the evaluation of injury to body tissue or for the detection of an inflammatory event somewhere in the body. CRP levels in serum are typically elevated in patients with arthritis or liver diseases such as Hepatitis A, Hepatitis B or biliary cirrhosis and after severe infections such as septic shock.

UREA , SERUM

Urea	19.00	mg/dL	16.6 - 48.5	Urease
BUN	8.88	mg/dL	7 - 18	Calculated

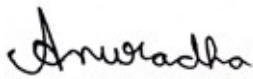
Interpretation Notes:-

High BUN levels may suggest that your kidneys aren't working as they should. However, even if your kidneys are working properly, you may have elevated BUN levels from the following:

- High-protein diet.
- Dehydration.
- Aging (infants and children have lower BUN levels).
- Certain medications, including carbamazepine, methotrexate and tetracycline.
- Burns.
- Blockage in your urinary system that prevents you from peeing.
- Stress.
- Heart attack.
- Gastrointestinal (GI) bleeding.

Low BUN levels aren't common. However, you may have low BUN levels from the following:

- Low-protein diet.
- Small body type.
- Overhydration (too much water in your body).


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- Liver disease.

CREATININE - SERUM , SERUM

Creatinine	0.59	mg/dL	Females: 0.50 - 0.90	Modified Jaffe Kinetic
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Interpretation Notes:-

High creatinine: Renal diseases and insufficiency with decreased glomerular filtration, urinary tract obstruction, reduced renal blood flow including congestive heart failure, shock, and dehydration; rhabdomyolysis can cause elevated serum creatinine.

Low creatinine: Small stature, debilitation, decreased muscle mass; some complex cases of severe hepatic disease can cause low serum creatinine levels. In advanced liver disease, low creatinine may result from decreased hepatic production of creatinine and inadequate dietary protein as well as reduced muscle mass.

ELECTROLYTE SERUM , SERUM

Sodium	138.40	mmol/L	135 - 150	ISE
Potassium	4.17	mmol/L	3.5 - 5.0	ISE
Chloride	97.10	mmol/L	94 - 110	ISE

Interpretation Notes:-

Too much or too little sodium can cause cells to malfunction. Lethargy, confusion, weakness, swelling, seizures, and coma are some symptoms that can occur with hyper - or hyponatremia.

Hypokalemia is most often seen when the body loses too much potassium from causes like vomiting, diarrhea, sweating, and medications like diuretics or laxatives. It is often seen in diabetic ketoacidosis, where potassium is excessively lost in the urine.

Hyperkalemia is a potentially life-threatening situation because it causes abnormal electrical conduction in the heart and potentially life-threatening heart rhythm problems. High potassium levels are most often associated with kidney failure, in which potassium levels build up and cannot be excreted in the urine.

Hypochloremia is an electrolyte imbalance and is indicated by a low level of chloride in the blood. Dehydration, fluid loss, or high levels of blood sodium may be noted and also experiencing other forms of fluid loss, such as diarrhea, or vomiting.

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MC-6941

Page 3 of 5

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GLUCOSE - FASTING (FBS) , NAF PLASMA

Fasting Glucose	87.70	mg/dL	70 - 110	Hexokinase
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Ref.for Biological Reference Intervals: American Diabetic Association.

Interpretation Notes:-

A blood glucose test measures the glucose levels in your blood. Glucose is a type of sugar. It is your body's main source of energy. Symptoms of high blood glucose levels include increased thirst, more frequent urination, Blurred vision, Fatigue, Wounds that are slow to heal. Symptoms of low blood glucose levels include Anxiety, Sweating, Trembling, Hunger and Confusion. Blood glucose test is required to check certain risk factors for diabetes. These include Being overweight, Lack of exercise, Family member with diabetes, High blood pressure, Heart disease.

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Page 4 of 5

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LIVER FUNCTION TEST (LFT) , SERUM

Total Bilirubin	0.52	mg/dL	< 1.2	Diazo
Direct Bilirubin	0.25	mg/dL	0 - 0.3	Diazo
Indirect Bilirubin	0.27	mg/dL	0.3 - 1.0	Calculated
SGOT / AST	51.40	U/L	< 40.0	IFCC with pyridoxal phosphate activation
SGPT / ALT	44.10	U/L	< 42.0	IFCC
SGOT (AST) : SGPT (ALT) Ratio	1.17	Ratio	Refer Interpretation Notes	Calculated
Alkaline Phosphatase	146.00	U/L	35 - 104	IFCC, PNPP AMP Buffer
Total Protein	7.73	g/dL	6.6 - 8.7	Biuret
Albumin	4.53	g/dL	3.5 - 5.2	Bromocresol green
Globulin	3.20	g/dL	2.5 - 4.5	Calculated
A/G Ratio	1.42	Ratio	1.0 - 2.1	Calculated
Gamma Glutamyl Transferase (GGT)	27.90	U/L	Men: 8 - 61 Women : 5 - 36	Enzymatic Colorimetric

Interpretation Notes:-

The most common liver tests include:

Liver enzymes test: Your liver enzymes include alkaline phosphatase (ALP), alanine transaminase (ALT), aspartate aminotransferase (AST) and gamma-glutamyl transferase (GGT). These are elevated when there's liver injury.

Total protein test: A total protein test measures levels of protein in your blood. Your liver makes protein, and low protein levels may indicate that your liver isn't functioning optimally.

Bilirubin test: Bilirubin is a waste product that liver deposits in bile. Possible diagnoses may include: Fatty liver disease, Toxic hepatitis, Autoimmune hepatitis, Viral hepatitis (A, B or C), Hemochromatosis, Wilson's disease, Alpha-1 antitrypsin deficiency, Primary biliary cholangitis (PBC), Cirrhosis. Liver cancer.

SGOT (AST) : SGPT (ALT) Ratio: A normal AST:ALT ratio should be <1. In patients with alcoholic liver disease, the AST:ALT ratio is >1 in 92% of patients, and >2 in 70%. AST:ALT scores >2 are, therefore, strongly suggestive of alcoholic liver disease and scores <1 more suggestive of NAFLD/NASH.

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*** End Of Report ***

Disclaimer:

1. It is presumed that the specimen belongs to the patient named or identified in the test request form. 2. Results pertain to the specimen submitted and all results are dependant on the quality of sample received in the laboratory. 3. The test results are for information and interpretation of the referring doctor or other medical professional who understands reporting units, reference ranges and limitations of terminologies. 4. Laboratory investigations help in arriving at a diagnosis and should be clinically correlated. 5. This report is not valid for medico-legal purposes. 6. Partial reproduction of this report is not permitted/not valid.

Page 5 of 5

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